

FINAL Male Fertility Issues

Sunday, 9 November 2025

9:23 am

F2F

^presents with ^

^New to me, presents

^Planned present for ^

^Patient and partner presenting for investigation of sub-fertility.

SUBJECTIVE

Duration: Trying to conceive for ^ months/years.

Partner's Details:

- Partner's age: ^yrs
- Known female factor issues (e.g., PMOS, endometriosis, tubal-blockage): Yes / No
- Partner's previous conceptions: Yes / No

Fertility History (Patient):

- Previous children (with current or previous partners): Yes / No, at age ^yrs

Sexual History:

- Frequency of intercourse: ^ times per week/month.
- Timing of intercourse (re: fertile window):
- Difficulties: Erectile dysfunction (present/absent), ejaculatory issues (premature, delayed, anejaculation), dyspareunia.
- Libido: Normal / Decreased / Increased.

Past Medical History:

Childhood:

- ^declines hx of cryptorchidism
- ^declines hx of scrotal surgery
- ^declines hx of hernia repair
- ^declines hx of mumps

Genitourinary:

- ^Declines hx of STIs (Chlamydia, Gonorrhoea)
- ^declines hx of Epididymo-orchitis / Prostatitis
- ^declines hx of Testicular/scrotal trauma
- ^declines vasectomy

Systemic Illness:

- ^no known hx of Diabetes Mellitus
- ^declines hx of chemotherapy / radiotherapy

Medications & Allergies

(mention if on TRT, 5-ARIs, spironolactone or sulfasalazine)

Social History:

Occupation:

- Exposure to: Heat / Pesticides / Solvents / Heavy metals / Radiation: Yes / No
- Details:

Substance Use:

- Smoking: Current / Ex / Never. (^ packs/day)
- Alcohol: ^ standard drinks per week.
- Recreational Drugs: ^Declines any
Marijuana: Yes / No, Cocaine: Yes / No
Opiates: Yes / No
Anabolic steroids (current or past): Yes / No

General Health:

- Diet and exercise:
- Recent high fevers: Yes / No
- Testicular heat exposure (e.g., saunas, hot tubs): Yes / No

Family History:

- Infertility (siblings, parents): Yes / No
- Genetic conditions (e.g., Cystic Fibrosis, Klinefelter): Yes / No

OBJECTIVE/ Examination

Vitals:

- BMI:
- Height
- Weight:

General appearance, secondary sexual characteristics (gynaecomastia, hair distribution, muscle mass):

Genital Examination:

- Penis: (e.g., meatal position)
- Testes (Scrotal Exam):
- Volume (Orchidometer): Left: _____ ml, Right: _____ ml (Normal >15ml)
 - Consistency: Firm / Soft / Other
- Epididymis: Tender / Non-tender / Cysts
- Vas Deferens: Present bilaterally / Absent (unilateral/bilateral)
- Varicocele:
 - Palpable: Yes / No
 - Present on Valsalva: Yes / No
- Other: (e.g., hydrocele, surgical scars)

ASSESSMENT

?Male factor sub-fertility

^There are no obvious predispositions on history.

^On examination, patient is well androgenised

PLAN

Investigations to consider:

- Semen Analysis (Initial):
 - [] Request 2x Semen Analysis (6-12 weeks apart).
 - *Instructions given: 2-7 days of abstinence, deliver to lab within 1 hour, keep at body temp.*

<https://www.snp.com.au/media/ghbjmts2/item-35170-semen-collection-202601.pdf> and
<https://www.snp.com.au/patients/patient-resources/pre-test-information/semn-collection/patient> (information sheet).

- Hormonal Profile (If SA abnormal or clinical suspicion):
 - FSH, LH, Testosterone (early morning, 8-10 am), Prolactin (if low T or low libido), SHBG
- Genetic (If severe oligo- or azoospermia):
 - Karyotype (Check for Klinefelter XXY)
 - Y-chromosome microdeletions
 - CFTR gene mutation (if vas deferens absent)
- Imaging:
 - Scrotal Ultrasound (if exam abnormal, varicocele suspected, or testicular mass).
- Other:
 - STI screen (if not recent)

Advice & further plan

Education:

- Discussed fertile window and advised regular intercourse (e.t., every 2-3 days).
- Reassurance provided.

Lifestyle Modifications:

- Advised smoking cessation.
- Advised reduction of alcohol.
- Advised cessation of illicit drugs (esp. marijuana, anabolic steroids).
- Weight management (if BMI high).
- Avoid testicular heat (e.g., loose underwear, avoid spas/saunas).

Medication Review:

- Reviewed current medications.
- Advised to stop [e.g., anabolic steroids] if applicable.

Supplements:

- Discussed limited evidence but low risk of antioxidants/folic acid.

Follow-up & Referral:

- [] Follow up to review Semen Analysis results.

Consider referral to Fertility Specialist / Urologist if:

- Abnormal Semen Analysis.
- Azoospermia or severe oligozoospermia.
- Suspected genetic cause.
- Significant examination finding (e.g., varicocele, absent vas).
- Female partner factors also present.

Patient information sheets:

1. <https://www.betterhealth.vic.gov.au/health/conditionsandtreatments/infertility-in-men>
 2. <https://www.thewomens.org.au/health-information/fertility-information/fertility-problems/male-infertility>
 3. https://www.ivf.com.au/sites/ivfa/files/2023-03/MFC05%20Male%20Fertility%20eBook%2017.03.23-LR_3.pdf
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